

August 20, 2026

BlueCross BlueShield of South Carolina
Provider Appeals Department
P.O. Box 100300
Columbia, SC 29202

RE: Formal Appeal of Claim Denial — Jerrell Gerlach (MRN MUSC3906137), Claim MUSC-D48A-3

Patient	Jerrell Gerlach (DOB 1939-01-27)
MRN	MUSC3906137
Member ID / Group	BCB447669083 / GRP-66199
Claim number	MUSC-D48A-3
Date of service	2026-04-22
Procedure billed	CPT 93458 — Left heart catheterization with coronary angiography
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$9,578.14
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-09-08

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal first-level reconsideration on behalf of patient Jerrell Gerlach (MRN: MUSC3906137, Member ID: BCB447669083, Group: GRP-66199) regarding the denial of Claim MUSC-D48A-3 for services rendered on April 22, 2026, at MUSC Health University Medical Center. BlueCross BlueShield of South Carolina denied the claim under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") with RARC N115, citing a Local Coverage Determination, and further noted that "documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and submits that the denial is the result of a coding discrepancy that, when corrected, fully supports the medical necessity of the procedure performed.

CLINICAL BACKGROUND

Mr. Gerlach is an 87-year-old male with a complex and longstanding cardiac history. He has carried a diagnosis of atrial fibrillation since 1994 and has been continuously managed with Verapamil Hydrochloride, Digoxin, and Warfarin Sodium since that time — a regimen consistent with rate control and anticoagulation therapy for chronic atrial fibrillation. His cardiac history is further documented by multiple electrical cardioversions performed in 2018, 2019, and 2020, as well as catheter ablations of cardiac tissue performed in 2017 and 2018, reflecting a pattern of recurrent, refractory atrial fibrillation requiring escalating intervention over many years. He also carries active diagnoses of anemia (D64.9) and prostate neoplasm, and is currently receiving Docetaxel and Leuprolide Acetate, agents associated with known cardiovascular effects. This clinical profile — a geriatric patient with decades of atrial fibrillation, prior ablations, prior cardioversions, active oncologic therapy, and anticoagulation — represents precisely the population for whom left heart catheterization with coronary angiography (CPT 93458) is a recognized and medically necessary diagnostic tool.

BASIS FOR APPEAL

The payer's denial rests on the assertion that the submitted documentation does not support medical necessity for the diagnosis reported. MUSC Health acknowledges that the primary diagnosis code submitted on this claim — J06.9, Acute upper respiratory infection, unspecified — does not reflect the clinical indication that prompted CPT 93458 on April 22, 2026. This represents a coding error on the submitted claim, not an absence of medical necessity. The correct primary diagnosis should reflect Mr. Gerlach's active, chronic atrial fibrillation and his broader cardiovascular condition, which are thoroughly documented in his longitudinal medical record. The secondary diagnosis of D64.9 (Anemia) was correctly reported and is clinically relevant given his active oncologic treatment. A corrected claim with the appropriate primary diagnosis code will be submitted concurrently with this appeal. We respectfully request that BlueCross BlueShield of South Carolina evaluate this appeal in conjunction with the corrected claim and the accompanying medical records, which clearly establish the cardiac indication for the procedure. The procedure was performed by Daniel R. Okafor, MD (NPI 1558493021) in an on-campus outpatient hospital setting, consistent with the acuity and complexity of Mr. Gerlach's condition.

REQUESTED ACTION

MUSC Health requests that BlueCross BlueShield of South Carolina overturn the denial of Claim MUSC-D48A-3 and reprocess the claim for payment upon receipt of the corrected claim reflecting the appropriate primary diagnosis. The billed amount for CPT 93458 is \$9,578.14, with a payer-allowed amount of \$6,750.78 on file. We ask that the claim be adjudicated at the contracted allowed amount in accordance with the terms of the BlueCross BlueShield of South Carolina Choice Plus plan. All supporting medical records documenting Mr. Gerlach's cardiac history and the clinical indication for this procedure are attached for review. This reconsideration is being submitted through My Insurance Manager (MIM) in advance of the appeal deadline of September 8, 2026, as required.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record